
The Year of Your Mid-Back

The crossroads of the body — and the most under-
treated area in most adults

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HEALING HANDS BY NATE

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Inner Circle Deep Cut — #3

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Introduction

AN INNER CIRCLE DEEP CUT. YEAR-LONG COMPANION for the area that influences almost everything else.

I'll tell you something I've said in other modules but it bears repeating in a deep cut format: if I could only work on one area of the body for the rest of my career, it'd be the mid-back. Everything connects through here. Everything.

After 12+ years on the table, I've come to see the mid-back as the keystone of the entire postural system. When it works, the rest of the body can work. When it's locked up — which it is in most adults who walk into my practice — almost every other complaint gets harder to resolve.

The mid-back is also the most under-treated area I see. People don't think to ask about it. They come in for neck pain or shoulder pain or low back pain, and the mid-back is the silent root cause. They've been getting massages on the symptom for years without anyone working the source.

This is the deep cut on the mid-back. Anatomy, patterns, self-assessment, daily practice, seasonal considerations, tools, and when to escalate. Year-long companion.

CHAPTER 1

How Your Mid-Back Actually Works

THE MID-BACK IS MORE COMPLEX THAN PEOPLE realize. It's not just "the part between the shoulder blades." It's a whole regional system that includes the thoracic spine, the rib cage, the connecting fascia, and the muscles that wrap all of it.

The thoracic spine

Twelve vertebrae from the base of your neck down to where your low back begins. The thoracic spine has three jobs:

Rotation — twisting your shoulders to look behind you, reaching across your body. **Extension** — arching backward, looking up. **Flexion** — rounding forward, slouching, curling up.

In a healthy adult, all three should happen freely throughout the day. In most adults, especially anyone with desk work or driving as part of their life, the thoracic spine is stuck in flexion. It rounds forward and stays there. Rotation is restricted. Extension is nearly impossible.

When the thoracic spine loses its motion, everything else has to compensate.

The rib cage

Twelve pairs of ribs attach to the thoracic vertebrae in the back and (most of them) to the sternum in the front. This creates a semi-rigid cage that protects your heart and lungs and provides attachment for breathing muscles.

The rib cage isn't supposed to be totally rigid. The ribs lift slightly with every inhale (your chest expands) and drop with every exhale. The joints where ribs meet vertebrae (costovertebral joints) and where ribs meet sternum (sternocostal joints) should glide smoothly.

When the mid-back is locked in flexion, the rib cage compresses. The ribs in the back compress against the spine; the ribs in the front splay slightly. Breathing becomes shallower because the cage doesn't expand fully. The whole upper body takes a more closed shape over time.

The fascia

The thoracolumbar fascia is a thick sheet of connective tissue that wraps the mid-back area, connecting it to the lats, the glutes, and the deeper layers of muscle. When this fascia is restricted, the whole mid-back loses its glide — and the restriction transmits

force across the back into the lats, shoulders, and even down to the hips.

The pectoral fascia in front does the opposite — when tight, pulls the chest forward, contributing to the mid-back flexion pattern.

The muscles

Several layers wrap around the thoracic spine:

Erector spinae — the long surface muscles. In the mid-back, they're the ones you can feel along either side of the spine between your shoulder blades.

Rhomboids — between your shoulder blades, connecting the inner edges of the shoulder blades to the spine. When weak (which they usually are in desk workers), the shoulder blades drift apart and round forward.

Middle and lower trapezius — large diamond-shaped muscle covering most of the upper and mid-back. The middle and lower fibers are critical for shoulder blade position. Usually weak in desk workers, leaving the upper traps to do too much.

Latissimus dorsi (lats) — large muscle from the lower back up to the upper arm. When tight, pulls the shoulder forward and restricts mid-back movement.

Intercostals — small muscles between the ribs. Often overlooked but important for rib cage mobility and breathing.

Multifidus — small deep stabilizers between thoracic vertebrae. Like in the lumbar spine, often weak in chronic mid-back issues.

This is more anatomy than you need to memorize. The takeaway: the mid-back is a complex system, and treating it requires addressing multiple layers, not just rubbing the surface muscles.

CHAPTER 2

The Five Mid-Back Patterns I See Every Week

Pattern 1 — Desk Flexion Lock

SYMPTOM: MID-BACK FEELS PERMANENTLY rounded. Can't fully arch backward. Reaching overhead feels restricted. Shallow breathing. Often paired with rounded shoulders.

What's happening: Years of forward-flexed posture (desk work, phone use, driving) have shortened the front-body tissue and the thoracic spine has lost its extension range. The mid-back is functionally stuck in slouch position even when you try to stand tall.

Who gets it: Almost everyone with a desk job. The patient demographic for this pattern is “modern adult.”

What helps: Open-book stretch (the single most important — see Bonus 4 deep dive), foam roller extension, cat-cow daily, doorframe stretch to address the front-body tightness too.

On the table: I do significant work on the thoracic erectors, the rhomboids, the lats, and the pec minor in front. The front-body work is critical — releasing the back without releasing the front means the chest pulls everything right back into flexion.

Pattern 2 — Tight Lats Restricting Rotation

Symptom: Can't twist your torso to look behind you. Reaching overhead causes pain or pulls on the side of the body. Sometimes the upper body feels “trapped” in one position.

What's happening: The lats — large muscles on the sides of your back — have become chronically tight. They restrict shoulder mobility AND mid-back rotation because they cross both areas.

Who gets it: People who lift weights without enough mobility work, people who carry heavy bags on one side, people with overdeveloped lats from sport (swimmers, climbers, rowers).

What helps: Lat-specific stretches (reach overhead, lean to the opposite side, hold 30 seconds), foam roller along the side of the back, and aggressive thoracic mobility work.

On the table: Lat release is uncomfortable but transformative. Once the lats let go, the mid-back rotation returns dramatically.

Pattern 3 — Breathing Dysfunction

Symptom: Can't take a deep breath. Mid-back feels tight specifically with breathing. Shoulders rise with inhale (instead of belly expanding). Constant background sense of "constricted."

What's happening: The diaphragm isn't doing its job. Accessory breathing muscles in the upper chest and neck have taken over. The mid-back tightens because the rib cage isn't moving the way it should — it's compressed in a "stress breathing" pattern.

Who gets it: People in chronic stress states. Often correlated with anxiety, history of trauma, or sustained high-stress life periods.

What helps: Diaphragmatic breathing practice (see Module 2), addressing the underlying stress where possible, vagus nerve work (Module 4), and direct rib cage mobility work.

On the table: I work the diaphragm directly (where accessible), the intercostals, the front of the ribs, and the breathing accessories in the neck. Often I have the client breathe deliberately during the work — direct nervous system retraining.

Pattern 4 — Scoliosis / Structural Asymmetry

Symptom: One-sided mid-back pain. Shoulders that aren't level. Hip on one side higher than the other. Pain that doesn't shift much with positioning changes.

What's happening: Structural asymmetry in the spine. Could be mild scoliosis (which most adults have to some degree), could be functional asymmetry from leg-length difference, sport-specific patterns, or old injuries.

Who gets it: Anyone with structural differences. Sometimes diagnosed in adolescence (scoliosis), sometimes never diagnosed but present.

What helps: Address what's addressable (functional asymmetries from posture or muscle imbalance) and accept what's not (structural curves that won't straighten). Targeted strengthening of the underactive side, stretching of the overworked side.

On the table: I can work with the asymmetric pattern, release the overworked tissue, and help maintain whatever range of motion you have. I cannot "fix" structural scoliosis — that's not what bodywork does.

When to see a doctor: Significant or progressing curves should be evaluated by an orthopedist. Severe scoliosis sometimes requires intervention.

Pattern 5 — Stuck Mid-Back from Old Trauma

Symptom: A specific area of the mid-back that just won't move. Sometimes very localized — “right there, between my shoulder blades.” Hands-on work helps temporarily but doesn't last.

What's happening: Old physical trauma (fall, car accident, sports injury) created a tissue restriction that hasn't fully resolved. Scar tissue, fascial adhesions, or nervous-system “guarding” of an area that was once injured.

Who gets it: Anyone with a significant injury history. The trauma might have been decades ago.

What helps: Patient, sustained bodywork over time. Sometimes specific manual therapy techniques (myofascial release, instrument-assisted soft tissue mobilization). Sometimes the area releases more during the period of nervous system safety (low stress, good sleep) than during direct work.

On the table: I work the area patiently. I don't try to “force” it. I often work the tissue around the stuck area first, then the area itself. Sometimes I'll release something stuck for 20+ years — but it takes multiple sessions and the right conditions.

CHAPTER 3

Self-Assessment

Question 1 — What's restricted?

- **Extension** (arching backward) → Pattern 1 most likely
- **Rotation** (twisting to look behind) → Pattern 2 or general flexion lock
- **Breathing depth** → Pattern 3
- **Asymmetric** (one side different from other) → Pattern 4

Question 2 — When does it hurt or feel restricted?

- All day, progressively worse → Pattern 1
- During lifting overhead → Pattern 2
- During emotional moments → Pattern 3
- One specific spot, regardless of position → Pattern 5

Question 3 — What does your breathing look like?

- Belly moves, chest doesn't rise much → Probably healthy
- Chest rises, belly barely moves → Pattern 3 likely

- Shoulders rise with each breath → Pattern 3 definitely

Question 4 — Have you had imaging?

- No abnormalities → Likely muscular (any pattern)
- Scoliosis diagnosed → Pattern 4
- Old injuries showing → Could be Pattern 5

Question 5 — Is it one-sided or both-sided?

- Both sides equally → Pattern 1, 2, or 3
 - One side significantly worse → Pattern 4 or 5
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CHAPTER 4

The Daily Mid-Back Maintenance Routine (10 Minutes)

Morning (3 minutes)

- Cat-cow on hands and knees, 10 slow reps
- Standing chest opener (hands clasped behind back, lift, hold 30 sec)
- Three deep belly breaths to finish

Midday (2 minutes)

IF YOU SIT AT A DESK, EVERY COUPLE OF HOURS:

- Stand up
- Seated thoracic rotation (cross arms over chest, rotate slowly to each side, 5 reps each)
- Doorframe stretch, 30 seconds

Evening (5 minutes)

- Open-book stretch, 5 reps each side (THE most important move — see Bonus 4 for the deep dive on form)

- Foam roller extension on mid-back, 30-60 seconds, work through 3 positions along the spine
 - Lat stretch (reach overhead, lean to opposite side, 30 sec each side)
 - Two minutes of diaphragmatic breathing to finish
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CHAPTER 5

Seasonal Mid-Back Issues

Spring (March-May)

PRIMARY ISSUE: SUDDEN ACTIVITY AFTER WINTER dormancy. Yard work and gardening activate mid-back muscles that haven't been working. Can trigger tweaks.

Prevention: Pre-activity warm-up that includes thoracic mobility (cat-cow, open-book). Don't skip it.

Summer (June-August)

Primary issue: AC-body cycling tightens upper back. Combined with summer activity load, the mid-back gets squeezed from both directions.

Prevention: Bonus 6 (AC reset) protocol. Cover the neck and upper back in cold environments.

Fall (September-November)

Primary issue: Cumulative summer load shows up. Mid-back often the slowest area to recover.

Prevention: Fall recovery protocol (Module 10). Extra mid-back mobility work in October.

Winter (December-February)

Primary issue: Cold-weather stiffness affects mid-back significantly. Reduced movement compounds the desk-flexion pattern.

Prevention: Daily morning warm-up. Hot shower on mid-back. Don't skip the daily open-book stretch.

CHAPTER 6

Tools, Props, and Products That Actually Help

Worth buying

FOAM ROLLER (\$20-40): THE SINGLE MOST USEFUL tool for mid-back self-care. Use as described in the daily routine.

Rolled-up bath towel (free): Substitute for foam roller. Tightly rolled, about 4-5 inches in diameter.

Lacrosse ball (\$5): For getting into the rhomboids and trapezius adhesions between the shoulder blades. Place against wall, lean into spot, hold.

Rice sock (homemade): Use on the mid-back for 10-15 minutes before stretching to soften the tissue.

A good chair (varies): If you sit for work, chair quality matters for mid-back. Doesn't need to be the most expensive — needs to provide reasonable lumbar support so you don't slouch (which compounds mid-back flexion).

Probably worth it for some people

Posture cueing devices (\$30-100): Wearables that beep when you slouch. Useful for awareness in some people, annoying and ignored by others. Try cheap before spending.

Backnobber or Theracane (\$20-30): Helpful for self-release of mid-back areas you can't reach. Some people love them, others find them awkward.

Don't waste your money on

"Posture corrector" braces: Generally weaken the muscles they're supposed to support. The muscles get dependent on the brace.

Most "back relief" gadgets sold on late-night TV: If it's marketed via dramatic infomercial, it's almost certainly not worth the price.

CHAPTER 7

When to See Me, When to See a Doctor, When to Wait

Wait (home care first)

- Generalized mid-back tightness from a known cause
- Stiffness that responds to the daily routine
- Mild discomfort that resolves within a few days

Come see me

- Mid-back issues lasting more than 2 weeks despite home care
- Recurring patterns
- Difficulty breathing fully or feeling “constricted” through the upper body
- The “stuck spot” that won’t release on its own
- Pre or post intense physical demands

See your doctor

- Sharp pain that didn’t respond to a specific injury

- Pain radiating around your chest in a band-like pattern (could indicate intercostal nerve issue or shingles)
- Difficulty breathing that's getting worse or accompanied by other symptoms
- Suspected scoliosis or significant asymmetry that wasn't there before
- Chest pain (could be cardiac — ER if severe)
- Pain associated with cough, fever, or other systemic symptoms

Coordinated care

FOR COMPLEX MID-BACK ISSUES:

- Physician for medical evaluation
 - Orthopedist for structural assessment if scoliosis or significant asymmetry
 - Pulmonologist if breathing dysfunction is severe
 - Physical therapist for graded thoracic mobility work
 - Bodyworker (me) for tissue release and pattern work
-

CHAPTER 8

Stories From the Table

“I came for my neck — why are you working my upper back?”

I GET THIS A LOT. CLIENT COMES IN SAYING their neck hurts. I assess and find the neck is downstream of mid-back lock. I work the mid-back primarily, with some neck work.

The client doesn’t feel “their neck got worked on” the way they expected. They tell me later they’re not sure if it helped.

Then a week passes. The neck doesn’t tighten back up the way it usually does after a session. They book another session. We do the same thing. The neck holds.

The lesson: treating where pain shows up isn’t the same as treating where pain comes from. Mid-back work feels less direct but produces more durable neck relief than neck work alone.

“I’ve gotten mid-back work for years and it doesn’t last”

If someone has been getting mid-back work for years and it doesn’t last, two things are usually happening:

1. The work is only addressing the tissue, not the patterns that re-create the tightness. Daily home practice is needed.
2. The front-body (pec minor, lats) isn’t being addressed. You can release the mid-back all day; if the front-body pulls everything forward again, the release won’t hold.

The fix: combine bodywork with daily mid-back mobility practice AND specific chest/front-body work.

“Can I just have you crack my back?”

No. That’s outside my scope of practice as a licensed massage therapist.

Spinal adjustment and joint manipulation are chiropractic territory — not bodywork. LMTs work on tissue: muscles, fascia, connective tissue. We don’t adjust joints. That’s a hard line, drawn by licensing and clinical scope. If someone offering you a “massage” tries to crack your back, that’s a problem — they’re working outside their scope.

What I CAN do is release the muscle and fascial tension that’s often the actual cause of feeling like “something needs to be cracked.” Most of the time,

what people interpret as “needing an adjustment” is really tight tissue restricting normal joint motion. Release the tissue, the joint often moves freely on its own without anyone manipulating it.

If you do need actual joint work — and sometimes that’s the right call — that’s a referral to a chiropractor. The two professions complement each other well: I handle the tissue side, they handle the joint side. Many clients benefit from working with both.

The fix: know what bodywork is and isn’t. If you want spinal manipulation, see a chiropractor. If you want soft-tissue work, see me. Both are valid. They’re different professions for a reason.

CHAPTER 9

What to Expect in a Year of Consistent Work

MONTH 1-2: AWARENESS OF HOW RESTRICTED your mid-back actually is. Some immediate relief from the daily routine. Notice how mid-back affects everything else.

Month 3-4: Daily routine becomes automatic. Easier deep breaths. Less neck and shoulder tension as the mid-back opens.

Month 5-6: Posture starts shifting. People notice you stand differently. Less effort required to maintain “good posture” because the structural restriction has eased.

Month 7-12: Continued refinement. The mid-back becomes a place you maintain rather than a place that’s chronically restricted. The downstream benefits (neck, shoulders, breathing, even mood) compound.

The mid-back is the area where consistent home practice produces the most dramatic structural change. Of all the regions, this is the one where 5-10

minutes a day actually rewires your default posture over months.

CHAPTER 10

Closing

THE MID-BACK IS THE KEYSTONE. WHEN IT works, everything else can work. When it doesn't, almost every other complaint becomes harder to resolve.

This document is your year-long companion. Reference the patterns when something flares. Use the daily routine as your baseline. Add the seasonal adjustments. Escalate to me or your doctor when appropriate.

If you internalize one thing from this entire deep cut: do the open-book stretch every day. Twice a day if you can. Five reps each side. The compounding effect on your mid-back, your breathing, your shoulders, your neck — even your mood — is significant over time.

The companion workbook has a pattern self-assessment, a 30-day mid-back tracker, and three reflection prompts to help you map your own mid-back's story over the year.

— Nate



This book was crafted by Nate Ratcliff,
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